



Havana Specialist Hospital, Board Committee Terms of Reference

Prepared for: The Board of Havana Specialist Hospital Limited

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Classification: Confidential

Five standing board committees. Each is a committee of the board, delegated authority to oversee and assure on the board's behalf, and reports to the board. Operational committees under the Afya-HSH Governance Committee (Quality Assurance, Hospital Manager, Clinical Administrator, Performance Review) are separate and report through that committee, not directly to the board. Composition names are proposals for the board to confirm.

Common provisions (apply to all five committees)

- **Establishment:** each committee is established by, and derives its authority from, the board. It has no powers beyond those delegated here and may not commit the company beyond its remit.
- **Secretary:** the Company Secretary (or delegate) services every committee, circulates papers at least five working days ahead, and keeps minutes and an action log.
- **Reporting:** each committee chair reports to the next board meeting on a fixed one-page template (key matters, decisions taken, items escalated, decisions requested).
- **Conflicts:** any member with a personal, family, or related-party interest in a matter declares it and recuses. This is recorded in the minutes and the conflicts register.
- **Access:** committees may obtain external professional advice at the company's expense within a board-approved limit, and have unrestricted access to management, records, and the Afya team.
- **Review:** terms of reference and committee effectiveness are reviewed annually.

1. Finance, Investment and Operations Committee

Purpose. Oversee financial performance, capital and investment decisions, and operational delivery. This is the board's performance committee, distinct from the independent assurance provided by Audit and Risk.

Authority (decision rights). Reviews and recommends the annual budget and business plan to the board; approves operational expenditure and investment within limits set in the Delegation of Authority schedule; recommends capital projects above its limit to the board.

Composition and chair. Co-chairs: Okeke Chukwuemeka (Family B) and Ugo Nwokoro (Family A). The committee is co-chaired by one director from each founding family so financial and operational oversight is shared and neither family dominates it. The co-chairs alternate or co-preside; a decision they cannot agree is referred to the full Board rather than settled by casting vote. Both chair as directors providing oversight, not in an executive line capacity, so it is not self-review. Members: Chief Odunukwe (investment), Dr Odulana or a further NED. In attendance (non-members): Hospital Manager / Head of Finance, Head of Operations, Afya finance lead. The Board Chair may attend but does not chair.

Quorum. Two members, at least one non-executive.



Cadence. Quarterly.

Core responsibilities.

- Management accounts, budget monitoring, cash and working capital.
- Capital and investment appraisal and post-investment review.
- Operational KPIs and service delivery against plan.
- Financial and commercial performance of the Afya arrangement (with independent assurance retained by Audit and Risk).
- Pricing, payer mix, and revenue performance.

Balance note: the money committee is co-chaired across both families; People and Remuneration is chaired by Family B; the Medical Director is Family A. Power at the top of the house is deliberately shared.

2. Audit and Risk Committee

Purpose. Provide independent assurance over financial reporting, internal control, internal and external audit, the enterprise risk register, and compliance. The committee that holds the finance function and management to account, and therefore sits apart from them.

Authority (decision rights). Recommends the external auditor's appointment, fee, and removal to the board; approves the internal audit plan; owns the enterprise risk register; reviews and clears related-party transactions within a board-approved threshold and refers larger ones to the board.

Composition and chair. Chair: Chief Charles Odunukwe (independent, financially literate; not the Board Chair and not the Finance chair, so assurance is independent of both). Members: Nwando Okeke (legal, compliance, related-party), Dr Odulana (recused from Consult for Africa related-party items). In attendance: Head of Finance, internal and external audit.

Quorum. Two members; the Board Chair and executives are not members and attend only by invitation.

Cadence. Quarterly, plus a dedicated audit-planning meeting. Five a year.

Core responsibilities.

- Annual financial statements, accounting judgements, and going-concern assessment.
- Internal control framework and the internal audit programme.
- External audit: appointment, independence, scope, findings, and management responses.
- The enterprise and financial risk register, and risk appetite recommendations to the board.
- Related-party transactions involving the families and Afya.
- The independent read on Afya's financial delivery.

3. Quality and Clinical Governance Committee

Purpose. Assure the board that the hospital delivers safe, high-quality clinical care and that clinical risk is identified and managed. (A governing body's oversight of quality and patient safety is a core, non-delegable board responsibility.)

Authority (decision rights). Sets and monitors clinical quality and safety standards; requires and reviews clinical incident investigations; recommends clinical policy, credentialing standards, and accreditation strategy to the board.



Composition and chair. Chair: Prof Oliver Ezechi (independent, clinical credibility; also Lead Independent Director). Members: Dr Ijeoma Iwuala (Medical Director, as executive clinical lead reporting in), plus Dr Hanson if appointed (nursing, accreditation). In attendance: Clinical Administrator, Quality Assurance lead.

Quorum. Two members, at least one non-executive.

Cadence. Quarterly.

Core responsibilities.

- Clinical quality metrics, patient safety, and outcomes.
- Incident and near-miss reporting, investigation, and learning.
- Consolidated review of the Tier 2 clinical functions (Clinical Administrator, Quality Assurance).
- Credentialing, clinical protocols, and scope-of-practice.
- Clinical risk (feeding the single enterprise risk register owned by Audit and Risk).
- Regulatory and accreditation readiness.

4. People and Remuneration Committee (with Nominations)

Purpose. Oversee board composition and succession, appointments, executive and director remuneration, board evaluation, workforce strategy, and conflicts management.

Authority (decision rights). Recommends director appointments, the chair, and committee memberships to the board through a formal, transparent nomination process; recommends the executive and director remuneration framework and the board sitting-allowance and participation policy; oversees the conflicts and related-party register.

Composition and chair. Chair: Nwando Okeke. Chairing this committee is given to the family that does not hold the Medical Director role, so the body that controls appointments and remuneration is a visible counterweight rather than another lever in one family's hands. Members: Prof Ezechi (Lead Independent Director), one executive director from the other family, Dr Odulana (recused from Consult for Africa fee matters).

Conflict guardrail. Because the chair is an executive director, a hard recusal rule applies: the chair and any member step out of decisions on their own and close family's remuneration, and the independent non-executives present carry those items. Nominations (composition and appointments) may be led by the chair; sensitive remuneration decisions are taken by the independents.

Quorum. Two members, at least one independent non-executive for any remuneration decision.

Cadence. Twice a year, plus as required for appointments.

Core responsibilities.

- The skills-gap review and appointment pipeline (including the payer-expertise and nursing decisions).
- Director induction, development, and annual board and chair evaluation (chair appraisal led by the Lead Independent Director).
- Executive and senior remuneration framework.
- The board sitting-allowance and participation policy.
- The conflicts of interest and related-party register.



5. Transformation and Digital Committee (time-boxed)

Purpose. Oversee the digital and operational transformation during the current build phase, in support of the growth and sustainability mandate.

Authority (decision rights). Recommends the technology and digital roadmap and major system selections to the board; oversees implementation and data governance within budget limits set in the Delegation of Authority schedule.

Composition and chair. Chair: Dr Debo Odulana (independent non-executive), so the committee provides oversight rather than being run by the executive delivering the work. Members: Head of Operations, Afya technology lead, and relevant executives in attendance; external adviser as needed.

Conflict guardrail. The chair recuses from any decision where Consult for Africa or CadreHealth could deliver or bid for the work.

Quorum. Two members, at least one non-executive.

Cadence. Monthly while transformation is active, then quarterly.

Core responsibilities.

- Technology and digital roadmap and prioritisation.
- Major system selection, implementation oversight, and benefits realisation.
- Data governance and information security.
- Digital patient experience.

Sunset. This committee is constituted with a defined review or sunset date. It is expected to migrate to a management (Tier 2) function once transformation stabilises, and should not become a permanent board committee by default.

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